

Experiential Reflection Essay: The Tinslee Lewis Case

Tinslee Breun Lewis was born with a rare congenital heart defect, Ebstein anomaly, as well as pulmonary atresia and chronic lung disease. Since birth, she had to undergo permanent life-sustaining medical care and had never left the cardiac intensive care unit. Even after the several open-heart surgeries her condition had not improved by any measurable clinical criterion. After her physicians had determined that further treatment was not only medically futile, but was actually and actively continuing to cause her pain. On October 30, 2019, the ethics committee of Cook Children convened - all twenty-two people unanimously agreed that, further life-sustaining treatment, was medically inappropriate. The hospital was trying to be relieved of life support devices ten days after advising the family under the controversial Texas Advance Directives Act (TADA). The mother of Tinslee, Trinity Lewis, strongly opposed this decision and, with legal assistance of the anti-abortion advocacy group Texas Right to Life, sought court injunctions to continue keeping her daughter on life support. The legal battle that followed lasted many years, involved political interference by the Texas Governor and Attorney General, a prompt legislative change, and ultimately ended in April 2022 when Tinslee, defying most clinical expectations, was discharged home after over three years of hospitalization.

The first thought that came to my mind when I first encountered the case of Tinslee Lewis was one of deep discomfort not so much because the situation was new in its concept, but because studying healthcare ethics had provided me with just enough of a framework to understand why all the parties involved could be simultaneously right and wrong. I was really disturbed not by the choice of one actor, but by the whole structure of the situation. The unanimous vote by the ethics committee has the ethical implication of not being an isolated opinion, but an institutional standard of care. And still, it is terribly disturbing to see the picture of a mother looking at the legal and medical system where her maternal instinct to protect her child when she is legally a minor is being violated. I was struck by what I can only describe as an epiphany about the concept of beneficence - I had learned it as 'acting in the best interest of the patient' but this case made me confront the fact that defining what is in the best interest of a non-verbal infant is not a clinical calculation in itself, but is also a moral, cultural, and relational judgment. I also, honestly speaking, was infuriated by the use of Tinslee and the weaponization of her plight into a political issue and a

legislative priority, which in my view was extremely disrespectful towards the grief of the family and the sincere concern and interest of the clinicians. What this experience has taught me is that healthcare ethics is not something that one takes in a course - it is a capacity that one has to continuously build.

Looking at the case of Tinslee Lewis from an ethics perspective in healthcare presents an interesting situation that is basically a clash of two principles of the field: non-maleficence (do no harm) and autonomy (in this instance, surrogate autonomy exercised by a parent on behalf of a minor). The argument of the medical team was based on non-maleficence, namely that it was not neutral to continue aggressive interventions on a terminally ill baby who was pharmacologically paralyzed, heavily sedated, and was said to be suffering with every routine procedure, in the name of hope. Clinically, the ethics committee's decision was sound, and at least in part, in line with the definition of medical futility. But I can't say the whole process in the hospital is fair. The TADA's structure, where the institution itself acts as a judge in a case that it is a party to, has serious due process concerns that even legal experts praised. The ten-day transfer window was an inadequate period for a family to work through a very complicated and emotionally difficult process. As an ethical value, justice requires fair and equitable treatment of all parties involved, and the procedures used in TADA failed to meet that mark in Tinslee's case. In parallel, it is important to take into account the principle of the patient's best interest — not the family's best interest — but it is an understandable and loving mother's decision who wants to continue with treatment. It isn't a bureaucratic footnote, it's a clinical consensus that speaks volumes that no other hospital would take Tinslee. I think the fault here is not so much one of intent, that is, there was no deliberate wickedness either on the part of the hospital or on the part of the mother, but one of process, communication and institutional design.

The timeline and the process of the ethics committee's decision and communication to the public, in my view, is the biggest mistake in the case of Tinslee Lewis. The 22 professionals that reached a unanimous clinical consensus is not a sudden development—these warning signs of medical futility have been there for months before the October 2019 committee meeting. I think that the hospital could have had a palliative care team and a patient advocate that was independent of the medical team earlier in Tinslee's hospitalization, not as a leadup to withdrawing, but as a constant means of communication between the medical team and Trinity Lewis. Palliative care is not about

giving up, it is the philosophy of care that places emphasis on the comfort of the patient and the emotional readiness of the family. If it had unfolded earlier, Trinity Lewis may not have felt so surprised and so much like a victim as a co-counselor to a broken heart. Also, I think that the hospital could have asked for the court's involvement in a proactive manner — not in a way that would take away Trinity Lewis' rights, but to have a neutral person involved early in the process to resolve due process issues that had become a problem and dragged out the litigation. The TADA's shortcomings in procedure were apparent prior to this case, and there was a clear belief on the hospital's part to provide something more than the statutory minimum of a dispute resolution process that was family focused. The legislation that ensued — which increased the notice period and changed the process for ethics committees — stands as justification for that stance, but it's too little, too late for Tinslee's family.

As a healthcare administrator who is involved in a situation like that of Tinslee Lewis, I would do everything possible to ensure institutional process and human dignity are not at odds; they are interdependent obligations. I'd make sure there was early and pro-active consultation with palliative care services in any case where a critically ill child is known to have a poor prognosis, and not as a final resort. Second, I would encourage others to develop an independent family liaison position within their institution who is a trained mediator, with no clinical involvement in the outcome and has the sole role of supporting families to understand, process and be a meaningful part of complex medical decisions. Third, I would argue that a medical futility determination would benefit from the inclusion of at least one "outside" member of the ethics committee from a bioethicist, a patient advocate or a community representative, to avoid the structural conflict of interest that exists when an institution decides its own case. Fourth, I would not leave my institution's policies at the statutory minimum required by law—the ethical minimum can be different, as the case of Tinslee Lewis makes abundantly clear. Finally, if I had to select a single word, it would be the need to have a difficult conversation early, to make sure families feel heard not managed, and to make every decision – no matter how painful – transparent, documented and viewed by an outside third party. We bear the responsibility as health care administrators to make these decisions fairly, but no one will ever find them easy.